Emergency Preparedness in Theatre — Full Notes

Extended study notes on recognising and responding to common perioperative emergencies.

General principles

- Call for help early — do not wait until a situation has fully escalated
- Know the location of the emergency trolley/crash cart before you need it
- Clear roles during an emergency reduce confusion — someone leads, others execute defined tasks
- Debrief after any emergency event, even one that resolved well

Anaphylaxis in theatre

- Common triggers include latex, antibiotics, and neuromuscular blocking agents
- Signs: hypotension, tachycardia, bronchospasm, skin changes — can be subtle under anaesthesia
- Immediate management: stop the likely trigger, call for help, adrenaline per protocol

Malignant hyperthermia

- A rare but life-threatening reaction to certain anaesthetic agents
- Signs: rapidly rising temperature, muscle rigidity, tachycardia, rising end-tidal CO2
- Management: stop triggering agents immediately, active cooling, dantrolene per protocol

Massive haemorrhage

- Early recognition of blood loss rate, not just total volume, drives the response
- Activate the massive transfusion protocol per local policy where blood loss is severe
- Communicate clearly and early with the blood bank and anaesthetic team

Fire in theatre

- Remove the ignition source and any fuel where safely possible
- Evacuate per the theatre's fire plan if the fire cannot be controlled immediately
- Prevention remains the priority — see the theatre safety precautions unit

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